

Gender: Female | Birth year: 1975 | WHO: 1

Tumor: Lung adenocarcinoma | Lesions: Liver, Lung | Stage: IV

Clinical summary

Relevant systemic treatment history2023-06 to 2025-01Osimertinib

Recent molecular results

Hartwig WGS (22-Feb-2025)

Molecular tissue of origin predictionLung: Non-small cell: LUAD (98%)  
Tumor mutational load / burdenTML 160 / TMB 14 mut/Mb  
Microsatellite (in)stabilityStable  
HR statusProficient (0)  
Driver mutationsEGFR C797S, EGFR L858R, KRAS G12C, KRAS G12D  
Amplified genesNone  
Deleted genesTP53 del  
Homozygously disrupted genesNone  
Gene fusionsMET(exon13)::MET(exon15) fusion  
Driver virusNone  
HLA-AHLA-A\*01:01, HLA-A\*02:01

Trial-relevant IHC results

PD-L1Score > 50%

Phase 2/3+ (or unknown phase) trials in NL that are open and potentially eligible (2 trials)

| Trial                                                         | Cohort                   | Molecular  | Sites                           | Warnings |
|---------------------------------------------------------------|--------------------------|------------|---------------------------------|----------|
| <a href="#">METC 04</a><br><a href="#">TEDR1</a><br>(Phase 2) | Lung cancer C797S cohort | EGFR C797S | NKI-AvL                         | None     |
| <a href="#">EGFR-C797S-TRIAL</a><br>(Phase 2)                 | EGFR C797S               | EGFR C797S | Elisabeth-TweeSteden Ziekenhuis |          |

Trials matched solely on molecular event and tumor type (no clinical data used) are shown in italicized, smaller font.

Phase 1/2 trials in NL that are open and potentially eligible (2 trials)

| Trial                                                            | Cohort                                   | Molecular                | Sites      | Warnings                                                    |
|------------------------------------------------------------------|------------------------------------------|--------------------------|------------|-------------------------------------------------------------|
| <a href="#">METC 02</a><br><a href="#">KAYRAS</a><br>(Phase 1/2) | Dose expansion - monotherapy - NSCLC     | KRAS G12D, PD-L1 >= 50.0 | Erasmus MC | Variant(s) G12D in KRAS but subclonal likelihood of > 50%   |
| <b>METC 01</b><br>IEMOEN<br>(Phase 1)                            | Dose escalation - monotherapy (no slots) | None                     |            | Has not exhausted SOC (at least platinum doublet remaining) |

International trials that are open and potentially eligible (2 trials)

| Trial                                        | Cohort     | Molecular  | Sites              |
|----------------------------------------------|------------|------------|--------------------|
| <a href="#">EGFR-BE (Phase 1)</a>            | EGFR L858R | EGFR L858R | Belgium: Brussels  |
| <a href="#">KRAS-G12C-TRIAL-DE (Phase 1)</a> | KRAS G12C  | KRAS G12C  | Germany: Stuttgart |

Trials in this table are matched solely on molecular event and tumor type (clinical data excluded).

Molecular Details

Hartwig WGS (EXAMPLE-LUNG-01-T, 22-Feb-2025)

General

| Purity | Ploidy | TML Status | TMB Status | MS Stability | HR Status      | DPYD                     | UGT1A1                   |
|--------|--------|------------|------------|--------------|----------------|--------------------------|--------------------------|
| 50%    | 2.3    | High (160) | High (14)  | Stable       | Proficient (0) | *1_HOM (Normal function) | *1_HOM (Normal function) |

Predicted tumor origin

|                                                       |     |
|-------------------------------------------------------|-----|
| 1. Lung: Non-small cell: LUAD                         |     |
| Combined prediction score                             | 98% |
| This score is calculated by combining information on: |     |
| (1) SNV types                                         | 60% |
| (2) SNV genomic localisation distribution             | 70% |
| (3) Driver genes and passenger characteristics        | 80% |

Other cohorts have a combined prediction of 2% or lower

Key drivers

| Type                        | Driver                    | Trials (Locations)  | Trials in Hartwig | Best evidence in External | Resistance in External |
|-----------------------------|---------------------------|---------------------|-------------------|---------------------------|------------------------|
| Mutation (gain of function) | EGFR L858R (2/4 copies)   |                     | NCT00000007       | Approved                  |                        |
| Mutation (gain of function) | EGFR C797S (1/4 copies)   | TEDR1 (NKI-AvL)     | NCT00000008       | Pre-clinical              |                        |
| Mutation (gain of function) | KRAS G12D (0.3/2 copies)* | KAYRAS (Erasmus MC) |                   |                           |                        |
| Mutation (gain of function) | KRAS G12C (0.3/2 copies)* |                     | NCT00000009       |                           |                        |
| Deletion                    | TP53 del                  |                     |                   |                           |                        |

The table continues on the next page

All results and data described in this report are for Research Use Only and have not been generated using a clinically validated and controlled procedure nor is it a validated medical device. The results should not be used for diagnostic or treatment purposes. No rights can be derived from the content of this report. Trials marked with asterisk (\*) were sourced from ClinicalTrials.gov on 17-Sep-2025. No modifications have been made to the ClinicalTrials.gov data. ACTIN structures trial information for matching and analytical purposes. Gene and variant annotations and related content are powered by Genomenon Cancer Knowledgebase (CKB). Lung cancer trial data may include content powered by Longkanker Onderzoek.

PATIENT  
EXAMPLE-LUNG-01  
REPORT DATE  
17-Sep-2025

| Type         | Driver                                                                                              | Trials (Locations) | Trials in Hartwig | Best evidence in External | Resistance in External |
|--------------|-----------------------------------------------------------------------------------------------------|--------------------|-------------------|---------------------------|------------------------|
| Known fusion | MET(exon13)::MET(exon15) fusion<br>Domain(s) kept: Tyrosine Kinase<br>Domain(s) lost: Juxtamembrane |                    |                   |                           |                        |

### Other drivers or relevant events

## Immunology

## IHC results

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Clinical Details

Clinical summary

|                                     |                    |                      |
|-------------------------------------|--------------------|----------------------|
| Relevant systemic treatment history | 2023-06 to 2025-01 | Osimertinib          |
| Relevant other oncological history  | None               |                      |
| Previous primary tumor              | None               |                      |
| Relevant non-oncological history    | 2023               | Rheumatoid arthritis |

Patient current details

|                                  |      |
|----------------------------------|------|
| Toxicities grade >= 2 or unknown | None |
| LVEF                             | 50%  |
| Known allergies                  | None |
| Recent surgeries                 | None |

Tumor details

|                    |                              |
|--------------------|------------------------------|
| Measurable disease | Yes                          |
| Lesions            | Liver, Lung                  |
| No lesions present | CNS, Brain, Bone, Lymph node |

Active medication details

| Medication      | Administration | Start date  | Stop date | Dosage         | Frequency  |
|-----------------|----------------|-------------|-----------|----------------|------------|
| St. John's Wort | Oral           | 01-Feb-2023 |           | 300 MILLIGRAMS | 1 / 2 DAYS |

Trial Matching Details

Trials and cohorts that are potentially eligible, but are closed (1 trial)

| Trial                                 | Cohort                       | Molecular | Sites | Warnings                                                    |
|---------------------------------------|------------------------------|-----------|-------|-------------------------------------------------------------|
| <b>METC 01</b><br>IEMOEN<br>(Phase 1) | Dose expansion - monotherapy | None      |       | Has not exhausted SOC (at least platinum doublet remaining) |

Trials and cohorts that are considered ineligible (4 cohorts from 3 trials)

| Trial                                      | Cohort                                                            | Molecular                | Ineligibility reasons                                       |
|--------------------------------------------|-------------------------------------------------------------------|--------------------------|-------------------------------------------------------------|
| <a href="#">METC 02 KAYRAS (Phase 1/2)</a> | Dose expansion - monotherapy - Colorectum                         | KRAS G12D, PD-L1 >= 50.0 | No colorectal cancer                                        |
| <b>METC 03</b><br>NO-SEE797ES              | Dose escalation - monotherapy                                     | EGFR C797S               | C797S in EGFR in canonical transcript                       |
| <b>METC 05</b><br>PICKME3CA                | <i>Applies to all cohorts below</i>                               | None                     | No PIK3CA activating mutation(s)                            |
|                                            | Dose expansion - monotherapy - NSCLC <i>(closed)</i>              |                          |                                                             |
|                                            | Dose expansion - monotherapy - Other cancer types <i>(closed)</i> |                          | Tumor belongs to DOID term(s) lung non-small cell carcinoma |

Trials and cohorts that are not evaluable or ignored (0 trials)

None